

ORTHOPEDIC SURGERY POST-OP NOTE

PATIENT	DOB	MRN	DATE OF SERVICE
Jonathan Davis	10/14/1995	948-20491-A	02/12/2024

SUBJECTIVE

Patient returns for 1-month follow-up of left transhumeral amputation. Reports wound is completely closed, but complains of intense 'electric shock' sensations and burning in his non-existent left hand (phantom limb pain). Rates overall phantom pain at 8/10, worse at night. Dislike touching the stump; says it triggers the shocks.

OBJECTIVE

Stump scar is fully healed; staples were removed during previous visit. No signs of infection, erythema, or fluctuance. Severe hypersensitivity to light palpation over the distal aspect of the stump. Tapping over the medial/posterior terminal stump triggers a severe radiation of pain into what feels like the absent ulnar and median distributions.

ASSESSMENT

1. Status post left transhumeral amputation, healing well. 2. Severe phantom limb pain and early post-amputation stump hypersensitivity.

PLAN

1. Increase Gabapentin to 600mg PO TID. 2. Refer to Hope Valley for stump desensitization therapy. 3. Recommend consultation with NextStep Prosthetics (CPO Robert Chen) for initial shrinker sock fitting.

Dr. Marcus Vance

Dr. Marcus Vance, MD, FACS — Orthopedic Trauma Surgery

NPI 1920491822 · Signed 02/12/2024

ORTHOPEDIC SURGERY - 5 MONTH CLINICAL NOTE

PATIENT	DOB	MRN	DATE OF SERVICE
Jonathan Davis	10/14/1995	948-20491-A	06/18/2024

SUBJECTIVE

Patient is back. He has completed several months of desensitization, but reports a localized, excruciatingly painful 'lump' on the distal medial aspect of his left stump. Pain is sharp and shooting, triggered even by his clothes rubbing against it. This is severely hindering his prosthetic fitting process with CPO Robert Chen.

OBJECTIVE

Left transhumeral stump shows normal outer skin contour, but deep palpation at the medial residual border reveals a highly tender, firm, mobile nodule measuring approximately 1.5 cm. Pressure on this nodule reproduces intense electrical shock down to the phantom hand.

ASSESSMENT

1. Post-amputation traumatic neuroma, left ulnar/median nerve stump. 2. Failed conservative management of stump hypersensitivity.

PLAN

Discussed options. Given the severity of the neuroma pain and its interference with prosthetic fabrication, we will proceed with surgical exploration, neuroma excision, and deep muscle transposition. Surgery scheduled for November 12, 2024, to allow further tissue maturation.

Dr. Marcus Vance

Dr. Marcus Vance, MD, FACS — Orthopedic Trauma Surgery

NPI 1920491822 · Signed 06/18/2024



OPERATIVE REPORT: LEFT RESIDUAL STUMP REVISION & NEUROMA EXCISION

PATIENT	DOB	MRN	DATE OF SERVICE
Jonathan Davis	10/14/1995	948-20491-A	11/12/2024

SURGEON: Dr. Marcus Vance, MD

PREOPERATIVE DIAGNOSIS: Symptomatic neuroma of left transhumeral residual limb.

PROCEDURE: Excision of neuroma, proximal resection of median and ulnar nerves with transposition into the triceps muscle.

OPERATIVE DETAILS:

The patient was anesthetized and positioned supine. Longitudinal incision was made over the medial aspect of the left residual limb, centered over the palpable nodule. Dissection down through subcutaneous tissue revealed a large, bulbous neuroma arising from the terminal ends of the ulnar and median nerves, bound in dense scar tissue.

The neuroma (1.8 cm x 1.4 cm) was carefully dissected free from the surrounding tissue. The nerve trunks were dissected proximally for approximately 5 cm. The nerves were then transected cleanly with a fresh scalpel blade. The proximal nerve stumps were then embedded deep into the triceps muscle belly using 5-0 epineural sutures to prevent recurrence.

The wound was thoroughly irrigated. Hemostasis was achieved. Skin closed with subcuticular sutures. Sterile dressing applied. Patient tolerated the procedure well.